

SPARTAN COACHING

DISCIPLINE | EMPATHY | STRATEGY

FACILITY-SPECIFIC SCRIPTS

Customized Conversation Guides by Healthcare Setting

Complete scripts for SNF directors of nursing, hospital discharge planners, physician offices, and assisted living administrators — each with openers, discovery questions, and closing language.

Facility-Specific Scripts

Tailoring Your Approach by Setting

SKILLED NURSING FACILITIES

Who You Are Meeting With

- Director of Nursing (DON): Clinical authority, staff supervisor, often the decision-maker for hospice partnerships
- Social Worker / Discharge Planner: Typically initiates the referral conversation with families
- Administrator: Business relationship owner, may need to be brought in for formal partnership agreements
- MDS Coordinator: Manages care planning and is often the first to identify declining residents

Key Pain Points in SNF Settings

- Staff burnout from managing complex end-of-life needs without clinical support
- After-hours coverage gaps when residents decline rapidly
- Family dissatisfaction and complaint management during difficult care periods
- CMS pressure on quality measures related to symptom management and avoidable hospitalizations

Opening Script for DON

"Hi [Name], I am [Your Name] with [Hospice Company]. I work with skilled nursing facilities to provide 24-hour clinical support for your most complex residents — especially those with CHF, COPD, dementia, and cancer who are at high risk of hospitalization. I have a few questions about how your team currently manages those residents. Do you have 10 minutes to talk?"

Discovery Questions for SNF Staff

1 The Burden Question

"When a resident is declining and the family is struggling to accept the situation, how does your team typically handle that conversation?"

2 The After-Hours Question

"How does your on-call team currently handle after-hours calls when a resident with serious illness needs clinical support?"

3 The Timing Question

"When residents do get referred to hospice, how long are they typically on service before they pass away?"

4 The Readmission Question

"Are there residents who are being hospitalized repeatedly for symptoms that hospice could manage here at the facility?"

Closing Language for SNF

"Based on what you have shared, I think there are a few specific situations where our clinical team could take a real burden off your staff — especially the after-hours support and the family communication piece. Would it make sense to schedule a time for me to meet with your social work team and walk through how our process works? No commitment, just a conversation to see if it is a good fit."

HOSPITAL DISCHARGE PLANNERS

Who You Are Meeting With

- Case Managers and Discharge Planners: Primary referral initiators in hospital settings
- Palliative Care Team Members: Clinical partners who often support hospice transitions
- Social Workers: Family communication leads and emotional support resources
- Unit Charge Nurses: Day-to-day point of contact for patient transitions

Key Pain Points in Hospital Settings

- Readmission penalties affecting reimbursement under value-based purchasing programs
- Length of stay pressures with beds needed for higher-acuity patients
- Inadequate community resources for complex patients after discharge
- Family resistance to accepting the severity of a loved one's condition

Opening Script for Case Manager

"Hi [Name], I am [Your Name] from [Hospice Company]. I know you are managing a full board right now, so I will be brief. I work with hospitals to help reduce readmissions for patients with CHF, COPD, and cancer by ensuring they transition to the right level of care at the right time. I have helped case managers identify patients for earlier hospice conversations and streamline the referral process when families are ready. Is there a better time this week for me to walk you through our process?"

Discovery Questions for Hospital Settings

1 The Readmission Question

"Which diagnosis categories are generating your highest readmission rates right now?"

2 The Timing Question

"When you identify a patient who might benefit from hospice, how early in the admission is that conversation typically happening?"

3 The Family Question

"When families are not ready to hear the hospice conversation, how does your team approach that?"

4 The Process Question

"What does your current referral process look like from initial conversation to actual discharge to hospice?"

Closing Language for Hospital

"Based on what you are describing, it sounds like timing is the biggest lever. I work with a few case managers at [similar hospital] who have found it helpful to have me do an initial family conversation alongside their social worker when there is resistance. It takes the pressure off your team and often moves things along more quickly. Could I show you how that works?"

PHYSICIAN OFFICES

Who You Are Meeting With

- Primary Care Physicians: High-volume referral potential with diverse patient populations
- Oncologists: Highest acuity patient population with late-stage disease trajectory
- Cardiologists: CHF and advanced cardiac patients with complex end-of-life needs

- Neurologists: ALS, Parkinson's, and stroke patients with long decline trajectories

Key Pain Points in Physician Settings

- Limited time for complex goals-of-care conversations during office visits
- Difficulty identifying the right moment to introduce the hospice conversation
- Feeling that referring to hospice means abandoning the patient
- Administrative burden of certification paperwork and ongoing documentation

Opening Script for Physician

"Dr. [Name], I know your time is extremely limited, so I appreciate the 5 minutes. I am [Your Name] with [Hospice Company]. We work alongside physicians like you to manage symptom burden and support families for your patients with serious illness, so that you can focus on the care decisions while we handle the day-to-day support piece. I would love to ask you two clinical questions about how you currently approach those situations. Is that okay?"

Discovery Questions for Physicians

1 The Clinical Question

"For patients with end-stage CHF or COPD, at what point do you typically begin introducing the comfort care conversation?"

2 The Outcome Question

"When patients do transition to hospice, do you typically stay involved in their care plan or does that relationship transfer?"

3 The Documentation Question

"The certification documentation for hospice can be time-intensive. What has your experience been with that process?"

Closing Language for Physician

"Many physicians tell me that they wish they had referred earlier because patients did so much better on hospice than expected. We can make the process very simple on your end — we handle the documentation, we provide the clinical updates, and you stay as involved in the patient's care as you want to be. Could we schedule a brief call with your nurse and me to talk through what that workflow would look like?"

ASSISTED LIVING COMMUNITIES

Opening Script for ALF Administrator

"Hi [Name], I am [Your Name] with [Hospice Company]. We partner with assisted living communities to help residents age in place and avoid unnecessary emergency transfers during serious illness. I work with your staff to provide the clinical support and family communication that keeps residents comfortable at home. I have a couple of questions about how your community currently handles residents who are medically declining. Do you have 10 minutes?"

Discovery Questions for ALF

1 The Transfer Question

"When a resident is declining significantly, what typically happens: do they stay in the community or transfer out?"

2 The Family Question

"Who typically has the conversation with the family about what options exist when a resident is near the end of life?"

3 The Staff Question

"How comfortable is your caregiving staff with managing residents who have significant symptom burden?"

ADAPTING TO THE ROOM

No script will work perfectly in every situation. Use these as starting frameworks, then listen carefully and adapt. The best hospice reps are skilled listeners first and skilled talkers second. When you are genuinely curious about a referral source's world, your questions feel natural and your conversations go deeper. Scripts become habits. Habits become instinct.